

Session Objectives

- Utilize DSM-V criteria to compare and contrast schizophrenia spectrum and other psychotic disorders.
- Describe the etiology and epidemiology of the schizophrenia spectrum and other psychotic disorders.
- Differentiate between primary psychotic disorders and secondary psychotic disorders (caused by medical problems)
- Describe the evidence-based assessments and treatment approaches for each.



Question 1

What are the key points in the case history?

- Entertaining internal dialogue
- Flat affect
- Appears to be distracted searching for someone/something
- Multiple previous hospitalization with “shots”
- THC positive on urine tox
- Lack of eye-contact
- Hallucinations and delusions: believes his loved ones poisoned him, hearing voices, etc.
- Prescribed an antipsychotic and stopped taking it recently



Schizophrenia



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DSM-V Criteria for Schizophrenia:

≥2 symptoms from this list, and **at least 1** must be a psychotic symptom*. Must be present for **at least 6 months**, and ≥ **1 month** of active symptoms

- Delusions*
- Hallucinations*
- Disorganized speech*
- Grossly disorganized or catatonic behavior
- Negative symptoms (flat affect, avolition, etc.)



Schizophrenia



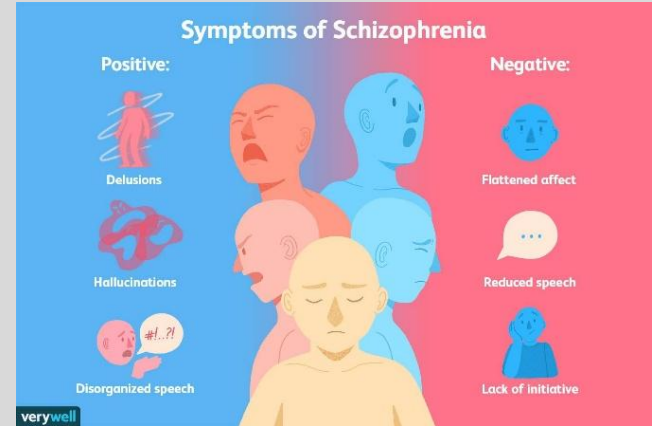
Positive Symptoms vs Negative Symptoms, what does that mean?

In psychology and psychiatry, “**positive**” means the addition of something that wasn’t there before. It does **not** mean that the symptoms are beneficial at all. In this case, **delusions, hallucinations, and disorganized speech**

Conversely, “**negative**” means taking something away. Thus, negative symptoms include **flattened affect, reduced speech, and lack of initiative/agency**

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Schizophrenia

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Psychotic symptoms include delusions, hallucinations or disorganized speech

- **Delusion:** Fixed, False belief (ex. Family poisoned me)
- **Hallucination:** Perceptions without external stimulus and can occur in any sensory modality (ex. hearing voices)
- **Disorganized speech:** Loosening of associations, derailment, or incoherence



Schizophrenia Spectrum and Other Psychotic Disorders: DSM-5 Selections.

Massachusetts General Hospital Comprehensive Clinical Psychiatry, Second Edition, Chapter twenty-eight, Clinical Features and Diagnosis.



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A quick note on **hallucinations**- they can affect ANY sensation, but some are more common in schizophrenia than others:

- **Auditory:** the most common in schizophrenia
- **Visual:** the second most common form in schizophrenia
- **Olfactory** is a common symptom in temporal lobe epilepsy, uncommon in schizophrenia
- **Tactile:** typical of cocaine or amphetamine use, alcohol withdrawal (ex. Feeling like bugs are crawling on skin)
- **Somatic:** involve a sensation from within the body and are obvious if bizarre. Ekbom's syndrome/delusional parasitosis is an example
- **Gustatory:** rare in schizophrenia





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A quick note on **delusions**: they are false, non-culturally sanctioned beliefs that are held with conviction, even in the face of overwhelming evidence to the contrary

- **Persecutory or paranoid**: most common in schizophrenia or delusional disorder. Belief person is being pursued, stalked, framed, or tricked. (cyberstalking/doxing example)
- **Ideas of reference**: thinks the news or paper are talking about or to them
- **Grandiose**: belief that some agency has taken control of their thoughts, feelings and/or behaviors.
- **Somatic**: part of the body is experienced as being diseased or malfunctioning
- **Nihilism**: exaggerated belief in the futility of everything.
- **Delusions of love**: aka erotomania, patients feel loved by another person, often of higher status, who is merely an innocent bystander
- **Jealousy**: suspecting someone of being unfaithful
- **Delusions of doubles** (aka **Capgras syndrome**): the patient believes that a family member or close friend has been an identically appearing double. More common in dementias.





Schizophrenia

Disorganized behavior can be described as odd, bizarre behavior such as smiling or laughing inappropriately or as if responding to internal stimuli

- Purposeless, ambivalent behavior or movements, and random, intermittent agitation
- Disheveled attire, unusual or inappropriate attire
- Disjointed speech patterns and behaviors reflect disorganized thinking: Perseveration, tangential speech, word-salad, use of neologisms





Delusional Disorder

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Delusional Disorder: defined by **1 or more** delusions lasting ≥ 1 **month(s)** and no other prominent psychotic symptoms (no hallucinations or disorganized speech)

Note: Hallucinations, if present, are mild and directly related to the delusion (e.g., smelling poison if convinced of being poisoned).

- The core feature is the presence of a well-systematized, encapsulated delusional belief
- Has an Insidious onset, frequently occurs in the elderly, and affects women more than men
- Frequently difficult to treat





Schizophreniform and Brief Psychotic Disorder

Similar criteria for symptoms as Schizophrenia, with the major difference being their timing!

Brief psychotic disorder: 1 positive symptom(s) lasting between 1 day and 1 month, usually stress-related. No negative symptoms present

Schizophreniform disorder: 2 symptoms lasting 1–6 months. Impaired functioning not required for diagnosis

Schizophrenia: Symptom onset ≥ 6 months prior to diagnosis; requires ≥ 1 month of active symptoms over the past 6 months.

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Schizophreniform and Brief Psychotic Disorder

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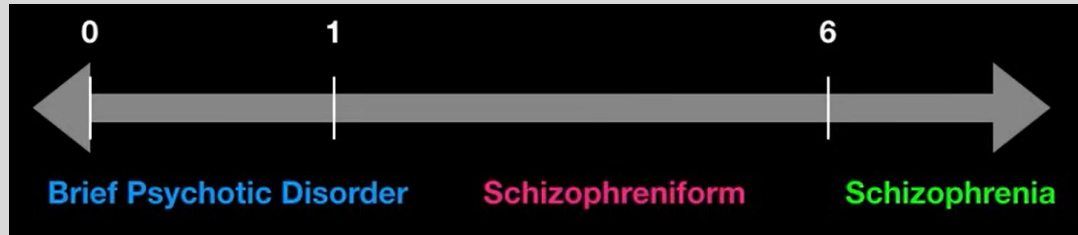
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Think of these three as being on a **continuum of duration and impact:**

Brief Psychotic = sudden onset, short-lived break with reality, full recovery, antipsychotics usually curative.

Schizophreniform = intermediate stage, uncertain prognosis,, also treated with antipsychotics, ~ 1/3 fully recover; most are later diagnosed with Schizophrenia

Schizophrenia = chronic, functionally impairing illness.



Clinical Pearl

Is there *another* psychiatric condition?

Clinical Features Suggesting a Secondary Psychotic Illness

- Symptoms of delirium
- Onset of first psychotic episode after age 40
- Rapid development of symptoms in a person without premorbid impairment
- Affect that is normal range and not blunted or flat
Predominance of hallucinations in modalities other than auditory
- Presence of a general medical condition with pathophysiology capable of causing or altering brain functioning
- Evidence of exposure to psychoactive substance capable of causing psychosis



Other Psychotic Disorders

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Schizoaffective Disorders

- Meet criteria for a **mood disorder** (either **unipolar** or **bipolar** type) **AND** criteria for **schizophrenia**.
- To differentiate from a mood disorder with psychotic features, patient must have ≥ 2 weeks of psychotic symptoms without a manic or depressive episode.
- Can have **Schizoaffective disorder, depressed type** (depression symptoms, SIG-E-CAPs) OR **Schizoaffective disorder, bipolar type** (manic symptoms, DIG-FAST)
- **Treatment:** SGAs for both subtypes, add antidepressants for depressed type, add mood stabilizers for bipolar type, ECT for refractory treatment



Test Taking Tip

- Test makers love to have students differentiate between **schizoaffective disorder**, **MDD with psychotic features**, and **Bipolar disorder with psychotic features**
- When answering questions the first part of the name is the primary **psychiatric disorder** while the second part of the name is the **secondary disorder**
- How do I know what is primary or secondary? Look at the question stem carefully to see which symptoms **occur by themselves for AT LEAST two weeks.**
- Ex. Patient presents with **psychotic symptoms** for at least two weeks and has **no** depressive symptoms. Depressive symptoms come and go. **Primary** disorder: schizoaffective, **secondary** disorder, depressed type

Schizoaffective Disorder

Schizo + Affective

Psychotic symptoms

Mood symptoms
(Depressed vs. Bipolar)

Schizoaffective Disorder, depressed type

Major Depressive Disorder, with psychotic features

Schizoaffective Disorder, bipolar type

Bipolar Disorder, with psychotic features





Other Psychotic Disorders

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Postpartum psychosis:

- Very rare, 0.1–0.2% incidence rate. Characterized by mood-congruent delusions, hallucinations, and thoughts of harming the baby or self.
- **Risk factors include:** first pregnancy, family history, bipolar disorder, psychotic disorder, recent medication change.
- **Treatment:** hospitalization and initiation of atypical antipsychotic; ECT for refractory cases.



Schizoid and Schizotypal

These are Type A personality disorders, NOT psychotic disorder (will go more in depth in a separate lecture later)

Schizoid Personality Disorder: Characterized by a profound detachment from social relationships and a restricted range of emotional expression. *Prefers* social withdrawal and solitary activities (vs avoidant), lack of interest in intimacy, limited emotional expression, indifferent to others' opinions (aloof).

Schizotypal Personality Disorder: On the schizophrenia spectrum. May include brief psychotic episodes (eg, delusions) that are less frequent and severe than in schizophrenia. Eccentric appearance, odd beliefs or magical thinking, interpersonal awkwardness. *May* like intimacy, but often find it difficult due to their symptoms and quirks.



Clinical Pearls

- Always look at the **teeth**. Poor dentition is quite common in **chronic schizophrenia**.
- Schizophrenia almost always leads to **nicotine** use
- **Hallucinations** in the **absence** of **negative symptoms** and especially **without** the other **positive symptoms** is almost always going to be due to a **substance or a medical cause**.
- Substance use is quite common in schizophrenia, often times used as a **coping skill** or way of managing the illness and not a cause. It can be difficult to differentiate.
- Trust your gut and hairs on the back of your neck. **ALWAYS** leave the interview room if you feel threatened or suddenly scared.



Summary Slide

- **Schizophrenia:** A chronic psychotic disorder lasting ≥ 6 months, with both positive (hallucinations, delusions, disorganized speech/behavior) and negative symptoms, causing significant functional impairment.
- **Brief Psychotic Disorder:** Sudden onset of positive psychotic symptoms lasting ≥ 1 day but < 1 month, followed by full return to baseline functioning.
- **Schizophreniform Disorder:** Psychotic symptoms similar to schizophrenia, lasting ≥ 1 month but < 6 months; functional decline is not required, and prognosis varies.
- **Schizoaffective Disorder – Depressed Type:** Features of schizophrenia (psychosis) along with major depressive episodes, with at least 2 weeks of psychotic symptoms occurring without mood symptoms.
- **Schizoaffective Disorder – Bipolar Type:** Schizophrenia features plus manic or mixed episodes (sometimes also depression), again requiring at least 2 weeks of psychosis without mood symptoms.



Summary Slide

- Postpartum Psychosis:** A rare, severe psychiatric emergency occurring within weeks after childbirth, marked by delusions, hallucinations, mood instability, and high risk of harm to self or infant.
- Delusional Disorder:** Presence of one or more fixed, non-bizarre delusions lasting ≥ 1 month, without other prominent psychotic symptoms or major functional impairment outside the delusional context.
- Schizoid Personality Disorder:** Lifelong pattern of detachment from social relationships and limited emotional expression, with preference for solitary activities and little interest in close connections.
- Schizotypal Personality Disorder:** Pervasive social and interpersonal deficits with cognitive/perceptual distortions (odd beliefs, magical thinking) and eccentric behavior, but without full psychosis.

